Metro Health Medical Center - Acute Care Pavilion

CLINICAL DOCUMENTATION — NARRATIVE EMERGENCY OBSERVATION NOTE V1

Patient: Jennifer Garcia | **MRN:** PAT-60006 | **DOB:** 1975-09-14 | **Gender:** Female | **Doc ID:** NARRATIVE_0006
Encounter Date: 2025-03-13 | **Discharge/Note Date:** 2025-03-16 | **Provider:** Dr. Allison Cameron, MD

EMERGENCY DEPARTMENT PRESENTATION

Patient Jennifer Garcia was triaged in the emergency department following acute onset of lightheadedness and palpitations earlier this morning. Initial hemodynamic parameters were reviewed and patient was placed on telemetry.

HISTORY OF PRESENT ILLNESS

The patient describes experiencing recurrent lightheadedness over the preceding 48 hours without syncope. Initial emergency workup revealed that initial bun was measured at 13.6 mg/dL. Point-of-care laboratory screening additionally noted alanine aminotransferase (alt) resulted at 27.6 U/L. Given these findings, the patient was transitioned to the Clinical Decision Unit for 24-hour observation.

OBSERVATION UNIT CLINICAL COURSE

Throughout the observation unit stay, continuous telemetry showed normal sinus rhythm without pause or tachyarrhythmia. The patient was administered Amlodipine at a dose of 2.5 mg orally once daily, which promptly alleviated acute symptoms. Follow-up protocol monitoring of sodium demonstrated 149.8 mmol/L (HIGH). Orthostatic blood pressures were negative upon ambulation.

DISCHARGE REGIMEN

Patient was deemed safe for outpatient discharge. The ongoing regimen includes Cephalexin 500 mg PO every 6 hours for 10 days as prescribed. Prescription counseling was performed by nursing prior to release.

PLAN & PRECAUTIONS

Resume normal daily activities as tolerated. Avoid strenuous exercise until cardiology follow-up in 1 to 2 weeks. Immediate emergency precautions detailed.